A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 May 2026

Dr Jamie Patel
Collingwood Medical

Dear Dr Patel,

Re: Mr James Johnson
DOB: 27/09/1991 Phone: 0459 937 894
Address: 53 Mater Street, COLLINGWOOD, VIC, 3066
Email: jcpjohnson20@gmail.com

Summary

- **Gastroscopy and colonoscopy excluding malignancy, inflammatory bowel disease, and coeliac disease.**
- **Internal haemorrhoids identified as the source of rectal bleeding.**
- **Iron deficiency likely multifactorial, related to diet and previous blood donation.**
- **Action required: Please monitor iron studies and support dietary iron intake/supplementation.**

I had the pleasure of reviewing James following his gastroscopy and colonoscopy, which were performed to investigate rectal bleeding and iron deficiency. I am pleased to report that the investigations were largely unremarkable and excluded significant pathology such as malignancy or inflammatory bowel disease.

Regarding his rectal bleeding, the colonoscopy identified internal haemorrhoids, which I believe are the clear cause. James noted that the bleeding had significantly lessened or stopped in the period leading up to the procedure. We discussed the mechanics of defaecation, specifically the need to avoid straining, maintain soft stools, and limit time spent on the toilet to prevent future haemorrhoidal flare-ups.

The gastroscopy revealed a small area of inflammation in the gastric antrum. However, biopsies were negative for Helicobacter pylori and there was no histological evidence of coeliac disease.

Impression and Plan I believe James's iron deficiency is likely explained by the combination of a previous blood donation and his relatively low dietary intake of red meat, which he currently consumes about once a week. While the small intestine remains a theoretical site for iron loss, the probability of finding a significant lesion in a young, healthy patient via a pill camera is very low—statistically less than one in a thousand in this context.

My recommendation is for James to focus on regular iron supplementation or increased dietary intake. I have also advised him to refrain from donating blood for the next 12 months to allow his stores to recover. I would be grateful if you could monitor his iron studies over the next few months. If his levels decline despite these measures, I have informed him that we will then proceed with a capsule endoscopy to evaluate the small bowel, though I suspect this will be unnecessary.

Kind regards

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

Johnson, James : HHS 176597

DOB: 27/09/1991 Age: 34 years

53 Mater St, Collingwood 3066

Results from 23/04/2026 ..... Episode# 26P300099 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B010579 **CLINICAL NOTES** Iron def. Gas = Erosion.? HP. Duo = N. Random colon = N. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Gastric bx". Two mucosal fragments measuring 1-2 mm. A1. 2. Specimen labelled "Duo bx". Five mucosal fragments measuring 1-4 mm. A1. 3. Specimen labelled "Random Colon bx". Five mucosal fragments measuring 1-7 mm. A1. **MICROSCOPIC DESCRIPTION** 1. The biopsy shows two fragments of non-specialised gastric mucosa with mildly increased plasma cells in the lamina propria. Other areas demonstrate some foveolar hyperplasia with mild convolution of gastric pits and mucin depletion. No obvious ulceration is noted. There is no active inflammation and a Helicobacter pylori immunostain is negative for organisms. There is no evidence of intestinal metaplasia, dysplasia or malignancy. 2. The biopsy shows fragments of duodenal mucosa with villous architecture and Brunner's glands. There is no villous blunting or increased intraepithelial lymphocytes. No active inflammation is seen. There is no evidence of ulceration, organisms, granuloma formation, dysplasia or malignancy. 3. The biopsy shows multiple fragments of colonic mucosa with preserved crypt architecture. The lamina propria contains a normal complement of chronic inflammatory cells. No active inflammation is seen. There is no subepithelial collagen plate thickening or increased intraepithelial lymphocytes. There is no evidence of ulceration, granuloma formation, organisms, dysplasia or malignancy. **PATHOLOGIST'S OPINION** **1. Gastric biopsy: Mild reactive gastropathy. 2. Duodenum biopsy: Within normal limits. 3. Random colon biopsy: Within normal limits.** Pathologist: Dr Joan Wang

All appointments and enquires

Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065